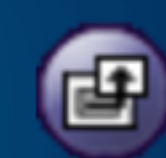


1. A 2-week-old newborn is brought for an initial examination. She was born at home to a 25-year-old primigravid woman following an uncomplicated pregnancy. Her parents say that it takes a long time to feed the newborn, and she usually does not finish her bottle. They are also concerned that she may be constipated. She is at the 50th percentile for length and weight and 90th percentile for head circumference. She has a hoarse cry. Vital signs are within normal limits. Examination shows large anterior and posterior fontanelles, a large protruding tongue, and a small umbilical hernia. The skin is dry and slightly mottled. There is mild hypotonia. Which of the following is most likely to have predicted these findings?

- ☐ A) Routine newborn screening
- ☐ B) Fetal monitoring
- ☐ C) Paternal karyotype analysis
- ☐ D) Prenatal ultrasonography
- ☐ E) Maternal glucose tolerance testing



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2. A 30-year-old woman comes to the physician because of a third episode of a Bartholin gland abscess over the past 5 years. Culture of the abscess cavity is most likely to show which of the following?

- ☐ A) *Bacteroides fragilis*
- ☐ B) *Chlamydia trachomatis*
- ☐ C) β -Hemolytic streptococcus
- ☐ D) *Neisseria gonorrhoeae*
- ☐ E) Mixed enteric and skin flora



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3. A 37-year-old woman comes to the physician because of a foul-smelling vaginal discharge for 10 days. She is sexually active with one partner. She had a bilateral tubal ligation 7 years ago. Examination shows a watery vaginal discharge with vulvar irritation. Which of the following is the most appropriate test to confirm the diagnosis?

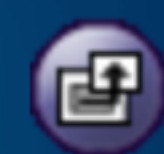
- ☐ A) Culture for *Candida albicans*
- ☐ B) DNA probe for *Chlamydia trachomatis* of the cervix
- ☐ C) Serologic testing for syphilis
- ☐ D) Vaginal culture for *Gardnerella vaginalis*
- ☐ E) Wet mount preparation



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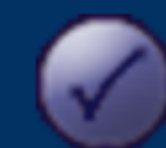
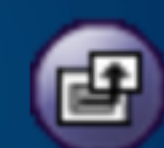
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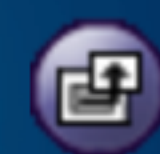
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4. A 25-year-old woman, gravida 1, para 1, comes to the physician because of almost continuous vaginal bleeding since delivery 5 days ago. She underwent a cesarean delivery because of a breech presentation at 40 weeks' gestation. She has had no difficulty breast-feeding. She is afebrile. Approximately 5 mL of old blood is visible in the vagina. The cervix is closed, and the uterus is firm and approximately 10 cm in length. Her hematocrit is 37%. Which of the following is the most appropriate next step in management?

- ☐ A) Observation
- ☐ B) Coagulation profile
- ☐ C) Administration of methylergonovine
- ☐ D) Administration of oxytocin
- ☐ E) Dilatation and curettage

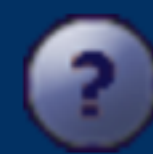
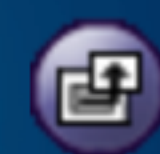


5. A 27-year-old woman at 7 weeks' gestation is admitted to the hospital for an appendectomy for a ruptured appendix. During the operation, one ovary is inadvertently removed due to extensive adhesions from previous operative procedures. Pathologic examination of an ovary specimen shows a corpus luteum cyst. Which of the following is the most appropriate next step in management?
- ☐ A) Estrogen supplementation until 10 weeks' gestation
 - ☐ B) Follicle-stimulating hormone therapy until 10 weeks' gestation
 - ☐ C) Progesterone supplementation until 10 weeks' gestation
 - ☐ D) Termination of pregnancy
 - ☐ E) No treatment is indicated



6. A previously healthy 18-year-old woman comes to the physician because of a 2-day history of vulvar pain. She is sexually active with three male partners, and they use condoms inconsistently. Examination shows a tender, nonindurated ulcer on the right labium majus that bleeds easily when touched. Which of the following is the most likely causal organism?

- ☐ A) *Calymmatobacterium granulomatis*
- ☐ B) *Chlamydia trachomatis*
- ☐ C) *Haemophilus ducreyi*
- ☐ D) Human papillomavirus
- ☐ E) *Treponema pallidum*



7. A previously healthy 27-year-old woman is brought to the emergency department 20 minutes after the sudden onset of loss of consciousness. She has a 1-day history of temperatures to 39.4°C (103°F), vomiting, watery diarrhea, and a rash over her face, trunk, and arms. Her last menstrual period started 5 days ago; she routinely uses tampons. Her temperature is 39.4°C (103°F), pulse is 125/min, respirations are 30/min, and her palpable systolic blood pressure is 70 mm Hg. Examination shows erythema of the conjunctivae and pharynx. Pelvic examination shows 10 to 15, 0.5-cm ulcers over the vaginal mucosa. Culture of a lesion scraping grows *Staphylococcus aureus*. This patient is at greatest risk for mortality from which of the following complications?
- ☐ A) Acute respiratory distress syndrome
 - ☐ B) Hemorrhage
 - ☐ C) Hepatic failure
 - ☐ D) Pulmonary embolism
 - ☐ E) Renal failure



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8. A 42-year-old nulligravid woman comes to the physician because she has been unable to conceive for the past year. She also says she has had hot flashes daily and vaginal dryness. Two years ago, she was diagnosed with cancer of the right breast; she underwent lumpectomy, sentinel node biopsy, and four cycles of chemotherapy followed by radiation therapy. There has been no evidence of recurrence. She has not had a menstrual period since beginning the second cycle of chemotherapy 18 months ago. She is in a monogamous relationship with a male partner and has frequent unprotected sexual intercourse. She is 163 cm (5 ft 4 in) tall and weighs 86 kg (190 lb); BMI is 33 kg/m². Her pulse is 80/min, respirations are 10/min, and blood pressure is 100/60 mm Hg. Examination of the right breast shows a well-healed lumpectomy scar. Examination of the left breast shows no abnormalities. The abdomen is soft and nontender. Pelvic examination shows vaginal atrophy with no vulvar, vaginal, or cervical lesions. The uterus is normal sized, and there are no adnexal masses. Which of the following is the most likely cause of this patient's infertility?

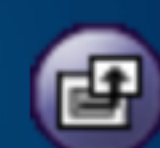
- ☐ A) Adrenal insufficiency
- ☐ B) Hyperprolactinemia
- ☐ C) Hypogonadism
- ☐ D) Hypothalamic-pituitary dysfunction
- ☐ E) Hypothyroidism



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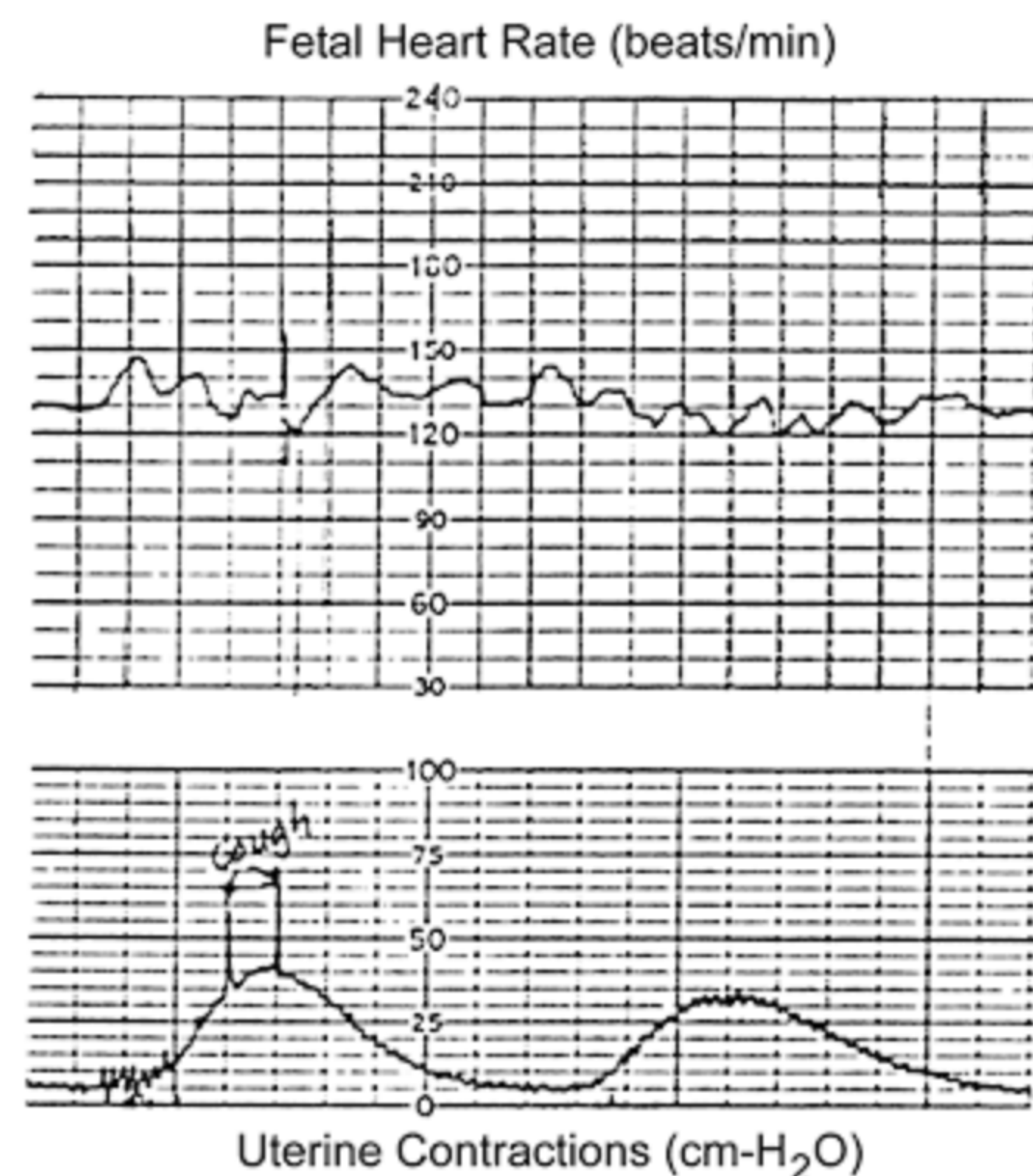
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9. A 32-year-old primigravid woman at 36 weeks' gestation comes for a routine prenatal visit. Her blood pressure is 122/70 mm Hg. Examination shows a fundus of 37 cm. Fetal heart tones are heard in the midabdomen, and the rate is 144/min. Deep tendon reflexes are brisk, and there is no pedal edema. The cervix is 50% effaced and 1 cm dilated; the vertex is at -1 station. During pelvic examination, the patient feels a sudden, dramatic fetal movement, and her membranes rupture. A large gush of clear fluid is seen. A fetal heart rate of 144/min is heard on auscultation. Cervical examination now shows no change in dilation or effacement. She begins to have painful contractions every 6 to 8 minutes that last 45 to 60 seconds. An electronic fetal monitor with an internal pressure catheter is applied. A fetal heart tracing is shown. Which of the following is the most appropriate next step in management?

- ☐ A) Observation and continued monitoring
- ☐ B) Fetal scalp blood pH sampling
- ☐ C) Administration of oxytocin
- ☐ D) Amnioinfusion
- ☐ E) Immediate cesarean delivery



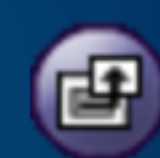
10. Four weeks after a total vaginal hysterectomy for cervical dysplasia, a 57-year-old woman comes for a follow-up visit. A pathology report confirms the presence of cervical intraepithelial neoplasia (CIN) 3 with all surgical margins clear of disease. Which of the following is the most appropriate recommendation for future screening for recurrent squamous cell intraepithelial neoplasia in this patient?
- ☐ A) Human papillomavirus testing in 3 months
 - ☐ B) Pap smears annually
 - ☐ C) Colposcopy of the vagina in 1 year
 - ☐ D) Random vaginal biopsy in 1 year
 - ☐ E) No further testing for this condition is necessary



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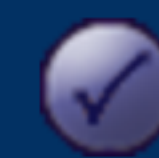
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11. A 24-year-old woman comes to the physician because of a 10-month history of breast tenderness that is worse just prior to the onset of menses. Her menses occur at regular 28-day intervals, and she is currently on day 26 of her menstrual cycle. Examination shows multiple, bilateral, firm cystic areas of the breasts. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) Reexamination after menses
- ☐ B) Cytology of expressed nipple discharge
- ☐ C) Mammography
- ☐ D) Aspiration of the cysts
- ☐ E) Biopsy of the cysts



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12. A previously healthy 47-year-old woman, gravida 3, para 3, comes to the physician because of a 2-week history of excessive vaginal bleeding with the passage of clots. During this time, she has had to change her sanitary pad every 2 hours. She has had occasional hot flashes during the past 6 months. Her menses previously occurred at regular 28-day intervals until 1 year ago. She now has 2 weeks of bleeding every 2 months. She has no history of serious illness and takes no medications. Six months ago, a Pap smear showed no abnormalities. She is 165 cm (5 ft 5 in) tall and weighs 66 kg (145 lb); BMI is 24 kg/m². Physical examination shows no abnormalities. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) Pap smear
- ☐ B) Measurement of serum luteinizing hormone concentration
- ☐ C) Measurement of serum testosterone concentration
- ☐ D) CT scan of the pelvis
- ☐ E) Endometrial biopsy



13. An 18-year-old woman is brought to the emergency department 2 hours after the onset of severe abdominal pain and nausea and vomiting. During a routine visit 2 weeks ago, pelvic examination showed an 8-cm right adnexal mass. Ultrasonography showed an 8-cm, mature cystic teratoma on the right ovary. Abdominal examination today shows severe rebound tenderness with guarding in the right lower quadrant. Which of the following is the most likely diagnosis?

- ☐ A) Acute appendicitis
- ☐ B) Ectopic pregnancy
- ☐ C) Ovarian torsion
- ☐ D) Pelvic inflammatory disease
- ☐ E) Renal calculi



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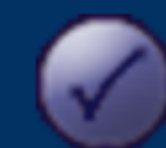
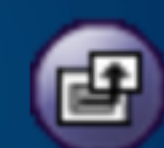
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14. A 27-year-old primigravid woman at 31 weeks' gestation comes to the physician because of a 3-day history of shortness of breath at rest. She has gestational diabetes treated with diet. She measures her blood glucose concentrations four times daily; her 1-hour postprandial concentrations have averaged 144 mg/dL. Ultrasonography at 18 weeks' gestation showed a fetus who had normal anatomy and was consistent in size with gestational age. The patient is Rh-negative and received Rh₀(D) immune globulin at 28 weeks' gestation; antibody screening was negative. Her pulse is 88/min, and respirations are 22/min. The lungs are clear to auscultation and percussion. Examination shows a fundal height of 44 cm. The cervix is 4 cm dilated and 50% effaced; the vertex is at -2 station. Which of the following is the most likely cause of this patient's symptoms?

- ☐ A) Anencephaly
- ☐ B) Incorrect gestational age
- ☐ C) Polyhydramnios
- ☐ D) Rh isoimmunization
- ☐ E) Twin gestation



15. A 27-year-old woman, gravida 3, para 2, comes to the physician for her first prenatal visit. Her last menstrual period was 12 weeks ago. Her previous pregnancies were uncomplicated. Examination shows a closed cervix and a uterus consistent in size with a 12-week gestation. Her blood group is AB, Rh-negative. Ultrasonography detects a fetal heart rate. Which of the following is the most appropriate next step in management?

- ☐ A) Glucose tolerance test
- ☐ B) Indirect antiglobulin (Coombs) test
- ☐ C) Measurement of hemoglobin A_{1c}
- ☐ D) Determination of Δ OD 450 in amniotic fluid
- ☐ E) Administration of Rh_o(D) immune globulin



16. A 22-year-old woman comes to the physician for counseling prior to conception. She feels well. She is recently married, and she and her husband wish to start a family immediately. She has a 10-year history of type 1 diabetes mellitus treated with an insulin pump. During the past 3 years, she has been hospitalized twice for ketoacidosis. She is 163 cm (5 ft 4 in) tall and weighs 50 kg (110 lb); BMI is 19 kg/m². Physical examination shows no abnormalities. Her serum glucose concentration is 180 mg/dL in the physician's office. Which of the following measures will have the greatest impact on the success of this patient's pregnancy?

- ☐ A) Adding an oral hypoglycemic to the regimen
- ☐ B) Exercise program
- ☐ C) Prenatal vitamin supplementation
- ☐ D) Tighter glucose control
- ☐ E) Weight gain



17. A 47-year-old woman comes for a routine health maintenance examination. She smoked one pack of cigarettes daily for 30 years, but she quit 1 year ago. She has had one lifetime sexual partner and has not been sexually active for 3 years. Pap smear and mammography showed normal findings 8 months ago. Her mother died of breast cancer at the age of 60 years. Physical and pelvic examinations show no abnormalities. Which of the following is the most appropriate screening test for this patient?

- ☐ A) Pap smear
- ☐ B) X-ray of the chest
- ☐ C) Mammography
- ☐ D) Colonoscopy
- ☐ E) No screening indicated



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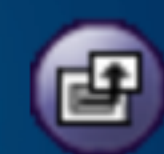
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18. A 30-year-old multigravid woman at 37 weeks' gestation is admitted in labor 6 hours after the onset of regular uterine contractions. Her vital signs are within normal limits. Physical examination shows no abnormalities. By abdominal palpation, her fetus is thought to be in breech presentation. Her amniotic membranes are intact. Her cervix is 2 cm long and 1 cm dilated; the presenting part is at -3 station but is difficult to palpate. Ultrasonography shows a double footling breech presentation. During labor, this patient is at greatest risk for which of the following complications?

- ☐ A) Chorioamnionitis
- ☐ B) Fetal shoulder dystocia
- ☐ C) Hemorrhage
- ☐ D) Prolapse of the fetal umbilical cord
- ☐ E) Uterine rupture



19. A 37-year-old woman, gravida 2, para 1, at 28 weeks' gestation comes to the physician for a routine prenatal visit. She feels well, and pregnancy has been uncomplicated. Amniocentesis for fetal karyotype at 16 weeks' gestation showed no abnormalities, and she received Rh_o(D) immune globulin. She has gained 8.2 kg (18 lb) during her pregnancy. Her blood pressure is 135/78 mm Hg. Fundal height is 29 cm. The fetus is in a longitudinal lie with a cephalic presentation. The fetal heart rate is 160/min. The patient's blood group is A, Rh-negative. Two days ago, serum anti-D antibody titer was negative. Which of the following is the most appropriate next step in management?
- ☐ A) Repeat serum anti-D antibody titer in 2 to 4 weeks
 - ☐ B) Kleihauer-Betke (acid elution) test
 - ☐ C) Administration of Rh_o(D) immune globulin
 - ☐ D) Amniocentesis for measurement of bilirubin concentration
 - ☐ E) Cordocentesis for measurement of fetal hemoglobin concentration



20. A 27-year-old woman at 22 weeks' gestation is admitted to the hospital because of fractures of right ribs 4–10 sustained in a motor vehicle collision. She is intubated. Her blood pressure is 120/80 mm Hg, and urine output is 50 mL/h. Fetal monitoring shows no fetal heart tones. Laboratory studies show:

Hematocrit	30%
Leukocyte count	9500/mm ³
Platelet count	70,000/mm ³
Prothrombin time	16 sec
Partial thromboplastin time	>100 sec
Plasma fibrinogen	50 mg/dL (N=200–400)

Fibrin split products are present in the serum at a 1:160 dilutional titer. Ultrasonography of the abdomen shows free fluid. Which of the following is the most appropriate next step?

- ☐ A) CT scan of the abdomen
- ☐ B) Exchange transfusion
- ☐ C) Platelet transfusion
- ☐ D) Transfusion of packed red blood cells
- ☐ E) Cesarean removal of the fetus

21. A 32-year-old primigravid woman at 26 weeks' gestation comes to the emergency department because of an 8-hour history of moderate abdominal pain and cramping. She has no history of serious illness or operative procedures. Her pregnancy had been uncomplicated. Vital signs are within normal limits. On pelvic examination, the cervix is 6 cm dilated and 100% effaced; the presenting part is at +1 station. Ultrasonography shows a fetus in a breech presentation and consistent in size with a 26-week gestation. The patient undergoes an emergent cesarean delivery. Twenty-four hours after delivery, the patient's temperature is 41°C (105.8°F), pulse is 130/min, respirations are 30/min, and blood pressure is 80/40 mm Hg. Cardiopulmonary examination shows no abnormalities. Abdominal examination shows moderate tenderness to palpation over the uterus. Urinalysis shows 2+ RBCs and trace leukocyte esterase. Which of the following is the most likely diagnosis?

- ☐ A) Pneumonia
- ☐ B) Puerperal sepsis
- ☐ C) Pyelonephritis
- ☐ D) Retained placental fragments
- ☐ E) Septic pelvic thrombophlebitis



22. A 62-year-old woman comes to the physician because of urinary incontinence for 4 weeks. Sometimes she is unable to reach a bathroom quickly enough after feeling the need to void. She recalls falling on wet grass 3 weeks ago but did not notice any injuries. She has gastroesophageal reflux disease and hypertension. Her medications include hydrochlorothiazide and famotidine. She stopped taking hydrochlorothiazide 7 days ago because she thought it was causing the incontinence, but the symptoms have continued. She takes no other medications. Her blood pressure is 137/92 mm Hg. Examination, including pelvic examination, shows no abnormalities. Which of the following is the most likely explanation for her incontinence?
- ☐ A) Adverse effect of medication
 - ☐ B) Loss of progesterone effect on the bladder mucosa
 - ☐ C) Obstruction of the bladder outlet
 - ☐ D) Spinal cord injury
 - ☐ E) Uninhibited bladder contractions



23. A 27-year-old woman, gravida 4, para 1, aborta 3, comes to the physician with her 31-year-old husband because of three first-trimester spontaneous abortions during the past 15 months. The patient received the diagnosis of each pregnancy by serum β -hCG testing and ultrasonography. After diagnosis, there was no evidence of embryonic viability on ultrasonography, and spontaneous abortion occurred at 7 weeks' gestation. Seven years ago, she had deep venous thrombosis of the left lower extremity while taking an oral contraceptive and was treated with anticoagulation therapy for 6 months. Physical examination of the patient and her husband shows no abnormalities. Which of the following is most likely to establish the cause of recurrent spontaneous abortion in this patient?
- ☐ A) Karyotype analysis of the patient and her husband
 - ☐ B) Karyotype analysis of subsequent abortus material
 - ☐ C) Serum antiphospholipid antibody assay
 - ☐ D) Hysterosalpingography
 - ☐ E) Endometrial biopsy during the luteal phase



24. A 32-year-old woman, gravida 1, para 1, is brought to the emergency department because of headaches for 4 days and slurred speech for 3 hours. She also has had fatigue and intermittent vaginal bleeding since delivering a healthy newborn at term 4 months ago; the bleeding has been heavy at times. Her blood pressure is 110/70 mm Hg. Examination shows a closed cervical os. Ultrasonography shows a slightly enlarged uterus with a small clot in the endometrial cavity. There are bilateral, 8-cm, cystic, ovarian masses. Neurologic examination shows right-sided weakness. Her hematocrit is 28%, and serum β -hCG concentration is 530,000 mIU/mL. Which of the following is the most likely diagnosis?
- ☐ A) Choriocarcinoma
 - ☐ B) Ectopic pregnancy
 - ☐ C) Germ cell ovarian tumor
 - ☐ D) Hydatidiform mole
 - ☐ E) Threatened abortion



25. A 37-year-old woman with a 2-week history of urinary frequency, urgency, and burning comes to the physician for a follow-up examination. One week ago, urinalysis showed 20–50 WBC/hpf with no protein, glucose, bacteria, ketones, or crystals. Urine dipstick showed trace blood. Her symptoms have not improved with 1 week of trimethoprim-sulfamethoxazole therapy. Pelvic examination shows urethral tenderness. The remainder of the examination shows no abnormalities. Urinalysis remains unchanged. In addition to urine culture, which of the following is the most appropriate next step in diagnosis?

- ☐ A) Cytologic examination of suprapubic urine aspirate
- ☐ B) Gram stain of the cervical mucus
- ☐ C) PPD skin test
- ☐ D) Throat culture
- ☐ E) Urethral culture for chlamydia



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26. A 30-year-old woman, gravida 1, para 1, has had fever and lower abdominal pain for 24 hours. Two days ago, she underwent a low transverse cesarean delivery because of failure to progress in labor after 18 hours. Her temperature is 39°C (102.2°F). The lungs are clear to auscultation. The incision is mildly tender and pink. The fundus is near the umbilicus and extremely tender. Which of the following organisms is the most likely cause of this infection?

- ☐ A) Gram-negative aerobes
- ☐ B) Gram-negative anaerobes
- ☐ C) Gram-positive aerobes
- ☐ D) Gram-positive anaerobes
- ☐ E) Mixed aerobes and anaerobes



27. A 37-year-old primigravid woman comes to the physician for a routine prenatal visit. She conceived 17 weeks ago by in vitro fertilization. Pregnancy has been uncomplicated. Physical examination shows no abnormalities. The uterus is consistent in size with a 17-week gestation. Her maternal serum α -fetoprotein concentration is 7.3 multiples of the median. Which of the following is the most likely fetal abnormality?

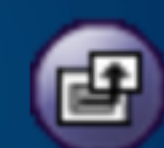
- ☐ A) Cystic fibrosis
- ☐ B) Down syndrome
- ☐ C) Myelomeningocele
- ☐ D) Tay-Sachs disease
- ☐ E) Ventricular septal defect



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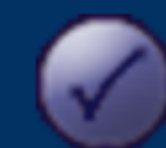
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28. Immediately after cesarean delivery because of abruptio placentae, a 25-year-old woman loses consciousness. She had an estimated blood loss of 1200 mL. Her pregnancy was uncomplicated. Her temperature is 37°C (98.6°F), pulse is 120/min, respirations are 26/min, and blood pressure is 60/40 mm Hg. Examination shows frothy pink sputum. Bilateral wheezes are heard on auscultation. The fundus is firm. Laboratory studies show:

Hemoglobin	7 g/dL
Leukocyte count	8000/mm ³
Platelet count	60,000/mm ³
Bleeding time	9 min
Prothrombin time	20 sec (INR=2)
Partial thromboplastin time	55 sec
Thrombin time	5 sec
Plasma fibrinogen	100 mg/dL (N=200–400)
Fibrin split products	80 µg/mL (N<10)

An x-ray of the chest shows pulmonary edema. An ECG shows tachycardia. Which of the following is the most likely diagnosis?

- ☐ A) Amniotic fluid embolism
- ☐ B) Eclampsia
- ☐ C) Hypovolemic shock
- ☐ D) Pulmonary embolism
- ☐ E) Septic shock

29. A 39-year-old woman, gravida 1, para 1, comes for a routine health maintenance examination. Menses have occurred at irregular intervals since menarche at the age of 14 years. Her last menstrual period occurred 6 months ago and was extremely heavy. She has declined to take hormones to regulate menses because she does not want to have a monthly menstrual period. Her only child was conceived 8 years ago after she was treated with clomiphene. She has had mild hirsutism since adolescence. Which of the following serum concentrations is most likely to be increased?

- ☐ A) Follicle-stimulating hormone
- ☐ B) Progesterone
- ☐ C) Testosterone
- ☐ D) Thyroid-stimulating hormone
- ☐ E) Thyroxine (T_4)



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30. A 16-year-old girl is brought to the physician by her mother because of an 18-month history of increasingly severe menstrual cramps associated with diarrhea and vomiting. The symptoms cause her to miss 2 days of school each month, and her school performance is beginning to decline. Menarche was at the age of 13 years. Menses occur at regular 28-day intervals. She is not sexually active. She is 165 cm (5 ft 5 in) tall and weighs 57 kg (125 lb); BMI is 21 kg/m². Physical examination shows no abnormalities. Pelvic examination shows normal-appearing external genitalia, vagina, and cervix. The uterus is small, mobile, and nontender. There are no adnexal masses or nodularity. Which of the following is the most likely cause of this patient's symptoms?
- ☐ A) Adenomyosis
 - ☐ B) Endometrial polyps
 - ☐ C) Endometriosis
 - ☐ D) Leiomyomata uteri
 - ☐ E) Primary dysmenorrhea



31. A previously healthy 23-year-old primigravid woman at 10 weeks' gestation has had nausea and vomiting for 4 weeks. She can tolerate small amounts of food intermittently but usually vomits 3 to 4 times daily. She is 157 cm (5 ft 2 in) tall; she has lost 1.8 kg (4 lb) and currently weighs 54 kg (119 lb). Ultrasonography shows a viable fetus consistent with a 10-week gestation. Which of the following is the most likely effect of her condition on the fetus?

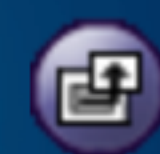
- ☐ A) Developmental delay
- ☐ B) Premature birth
- ☐ C) Pulmonary hypoplasia
- ☐ D) Spontaneous abortion
- ☐ E) No significant adverse effect



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32. A 27-year-old primigravid woman at 8 weeks' gestation comes to the physician because of a 1-day history of a migraine. She has a history of migraines treated with sumatriptan. She reports that she has not taken any pain relief medication for her headache because of concern regarding possible harm to her fetus. Vital signs are within normal limits. Physical examination shows a uterus consistent in size with an 8-week gestation. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Acetaminophen
- ☐ B) Butalbital
- ☐ C) Ergotamine
- ☐ D) Meperidine
- ☐ E) Sumatriptan



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33. A 17-year-old primigravid patient has been in labor for 15 hours; she has made no progress for the past 3 hours. The cervix is 5 cm dilated and 80% effaced; the vertex is at +2 station. Contractions occur every 5 minutes. Assuming a normal-sized full-term fetus and a clinically adequate pelvis, which of the following is the most appropriate next step?

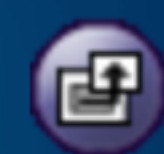
- ☐ A) Augmentation of labor with oxytocin
- ☐ B) Cesarean delivery for failure to progress
- ☐ C) Intramuscular sedation for 2 hours of rest
- ☐ D) Intravenous hydration
- ☐ E) X-ray pelvimetry



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34. A 23-year-old woman, gravida 1, para 1, comes to the physician because of increasingly severe pain of the left breast over the past 3 days. She now describes the pain as excruciating. Five days ago, she delivered a 3700-g (8-lb 3-oz) newborn vaginally after an uncomplicated labor. Her pregnancy was complicated by oligohydramnios, and labor was induced at 39 weeks' gestation. Her postpartum course has been uncomplicated. She is breast-feeding. Her temperature is 37.5°C (99.5°F). Examination shows a tender, edematous area on the lateral aspect of the left breast that is not erythematous or warm. Which of the following is the most appropriate next step in management?
- ☐ A) Application of heat to the area
 - ☐ B) Aminoglycoside therapy
 - ☐ C) Dicloxacillin therapy
 - ☐ D) Needle aspiration and culture
 - ☐ E) Incision and drainage



35. A previously healthy 47-year-old woman comes to the physician 3 weeks after she noted a lump in her right breast during self-examination. Physical examination shows a 3-cm mass in the upper outer quadrant of the right breast. Mammography shows no abnormalities. Ultrasonography shows a cystic component; fine-needle aspiration biopsy yields cloudy straw-colored fluid. The mass is still present after aspiration. Which of the following is the most appropriate next step in management?

- ☐ A) Observation
- ☐ B) Repeat examination in 6 weeks
- ☐ C) Repeat examination in 1 year
- ☐ D) Repeat mammography
- ☐ E) Thermography
- ☐ F) Biopsy of the mass



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36. A 23-year-old woman, gravida 2, para 1, is admitted in labor at term. Her pregnancy has been uncomplicated. The cervix is 4 cm dilated and 75% effaced; the vertex is at -1 station. Two hours later, the cervix is 5 cm dilated and 75% effaced; the vertex is at 0 station. Fetal heart rate is 140/min. Four hours later, the cervix is 6 cm dilated; the rest of the examination is unchanged. Which of the following is the most likely diagnosis?

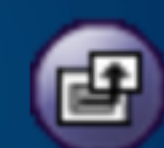
- ☐ A) Prolonged latent phase of labor
- ☐ B) Prolonged second stage of labor
- ☐ C) Protracted active phase of labor
- ☐ D) Protraction of descent
- ☐ E) Normal labor



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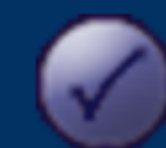
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37. A 32-year-old woman, gravida 4, para 3, at 38 weeks' gestation is admitted to the hospital 8 hours after premature rupture of membranes. Pregnancy had been uncomplicated. Previous pregnancies were uncomplicated with spontaneous vaginal delivery of 3430-g (7-lb 9-oz), 3629-g (8-lb), and 3770-g (8-lb 5-oz) newborns at 37 to 39 weeks' gestation. She has no history of serious illness. Her only medication is a prenatal vitamin. Her temperature is 37.2°C (99°F), pulse is 80/min, respirations are 16/min, and blood pressure is 112/65 mm Hg. The cervix is 1 cm dilated and 80% effaced. Labor augmentation with oxytocin is begun and results in contractions 2 to 3 minutes apart. The cervix is now 8 cm dilated and 100% effaced; the vertex is at +2 station. Her temperature now is 39°C (102.2°F), pulse is 110/min, respirations are 22/min, and blood pressure is 105/58 mm Hg. The baseline fetal heart rate is 175/min with mild variable decelerations. The uterus is tender. Which of the following is the most appropriate next step in management?

- ☐ A) Antibiotic therapy
- ☐ B) Increased oxytocin therapy
- ☐ C) Forceps delivery
- ☐ D) Vacuum extraction
- ☐ E) Immediate cesarean delivery



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38. A 67-year-old woman, gravida 3, para 3, comes to the physician because of a 1-month history of a rapidly increasing lower abdominal mass. Twenty years ago, she underwent radiation therapy of the pelvis for early cervical cancer; the uterus was not removed. She has been receiving estrogen and progesterone replacement therapy since then. She has smoked two packs of cigarettes daily for 40 years. Examination shows a normal-appearing vulva, vagina, and cervix. The uterus measures 16 cm. She undergoes exploratory laparotomy, total abdominal hysterectomy, and bilateral salpingo-oophorectomy. Pathologic evaluation shows a uterine sarcoma. Which of the following is the most likely underlying cause of this pathologic finding?
- ☐ A) Cigarette smoking
 - ☐ B) Excessive estrogen
 - ☐ C) Genetic predisposition
 - ☐ D) Human papillomavirus infection
 - ☐ E) Pelvic radiation therapy



39. A 77-year-old woman, gravida 7, para 5, aborta 2, comes to the physician because of a mass protruding from the vagina for 1 week; she feels something moving within the mass. She has also had severe low back pain for 6 months. Eight years ago, she underwent a vaginal hysterectomy and anterior and posterior colporrhaphy for treatment of uterovaginal prolapse. Which of the following is the most likely diagnosis?

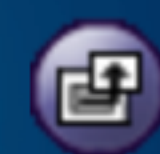
- ☐ A) Cystocele
- ☐ B) Enterocele
- ☐ C) Femoral hernia
- ☐ D) Indirect inguinal hernia
- ☐ E) Sigmoid intussusception
- ☐ F) Urethrocele



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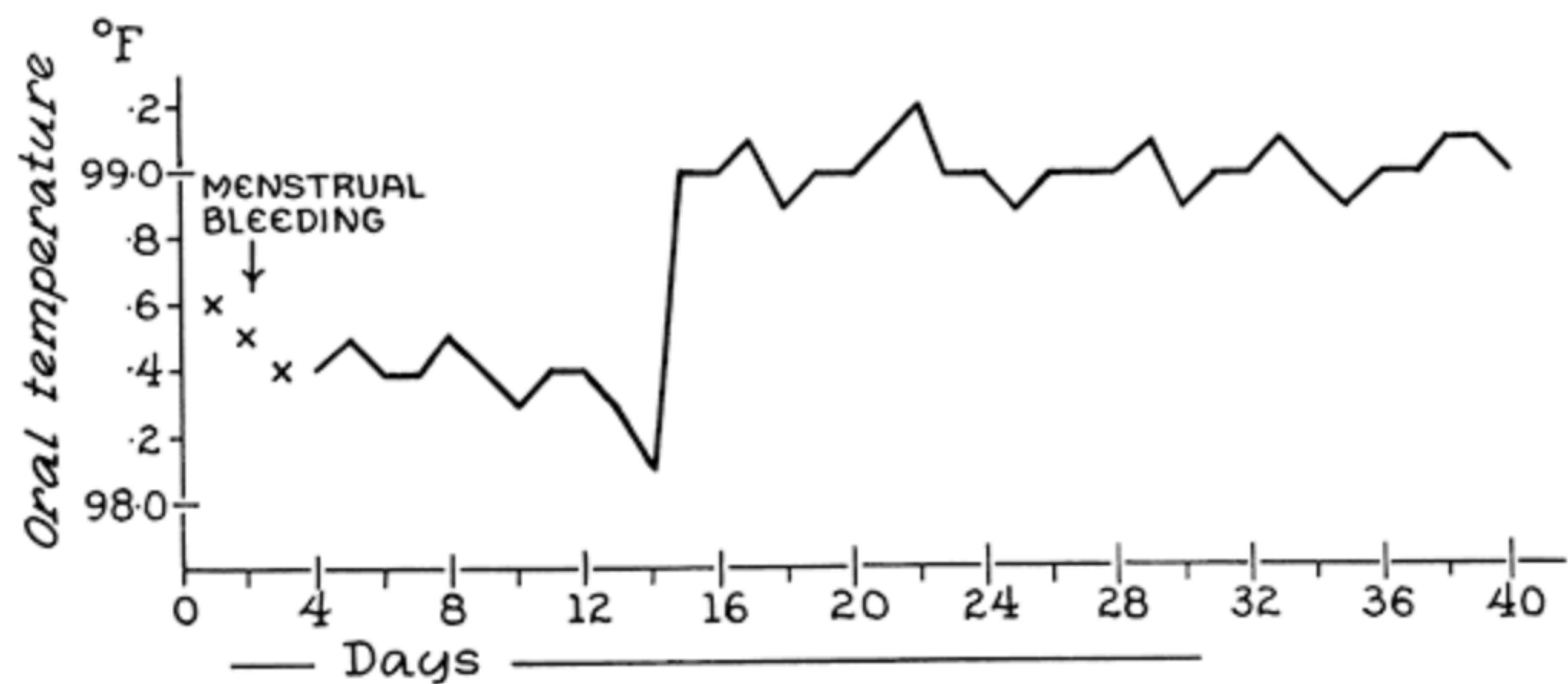
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40. A healthy 27-year-old woman, gravida 3, para 2, at 32 weeks' gestation comes to the physician for a routine prenatal visit. She says that she feels well and has not had any uterine contractions. She reports that the baby has been very active. The patient has had a 1-kg (2-lb) weight gain since her last visit 2 weeks ago. She does not drink alcohol and quit smoking after she learned she was pregnant. She works as a bartender. Her blood pressure is 120/70 mm Hg. Examination shows a fundal height of 28 cm. Fetal heart tones are 150/min. Leopold maneuvers confirm a cephalic presentation. Which of the following is the most appropriate next step in management?
- ☐ A) Follow-up visit in 2 weeks
 - ☐ B) Fetal ultrasonography
 - ☐ C) Induction of labor
 - ☐ D) Percutaneous umbilical blood sampling
 - ☐ E) Amniocentesis
 - ☐ F) Cesarean delivery





41. The basal body temperature graph shown was obtained from a 22-year-old nulligravid woman who has been unable to conceive for 18 months. Menses occur at regular 28-day intervals. Which of the following hormones is the most likely cause of the increase in basal body temperature on the 15th day?
- ☐ A) Estrogen
 - ☐ B) Follicle-stimulating hormone
 - ☐ C) Human chorionic gonadotropin
 - ☐ D) Luteinizing hormone
 - ☐ E) Progesterone

42. A 32-year-old woman comes to the physician for a follow-up examination 4 months after beginning a trial of oral contraceptive therapy for menometrorrhagia. Her symptoms have improved. Menses now occur at regular 28- to 30-day intervals and last 3 days. Her last menstrual period was 2 weeks ago. She is otherwise healthy and takes no other medications. Physical examination, including pelvic examination, shows no abnormalities. Which of the following is the most likely explanation for the improvement in this patient's bleeding?
- ☐ A) Enhanced progesterone secretion by the corpus luteum
 - ☐ B) Increased luteinizing hormone concentration
 - ☐ C) Suppression of uterine adenomyosis
 - ☐ D) Synchronization of the endometrium
 - ☐ E) Thickening of the cervical mucus



43. A 66-year-old woman is admitted to the hospital because of a 2-day history of decreased level of consciousness and dyspnea. Two years ago, she was diagnosed with stage IV epithelial ovarian cancer. The patient initially responded to surgical debulking and chemotherapy, but she has not responded to the past three chemotherapy regimens. Her last CT scan of the abdomen showed an increase in peritoneal tumor burden. The patient does not have an advance directive. She appears thin and is in moderate respiratory distress. She is unconscious and does not respond to verbal stimuli. Her temperature is 37.7°C (99.9°F), pulse is 110/min, respirations are 24/min, and blood pressure is 80/50 mm Hg. Breathing is labored, and breath sounds are decreased in the lower lung fields bilaterally. Her serum CA 125 concentration is 1236 U/mL (N<35); 1 month ago, it was 247 U/mL. The physician requests a family meeting with her two daughters. Which of the following is the most appropriate initial statement by the physician?
- ☐ A) "Do you want your mother to be an organ donor?"
 - ☐ B) "What do you think is happening with your mother?"
 - ☐ C) "Who is in charge of making decisions about your mother's care?"
 - ☐ D) "Would you like your mother to be put on life support?"
 - ☐ E) "Your mother will not live through the night."



44. An 82-year-old woman with dementia, Alzheimer type, is brought to the physician by her daughter because of blood on her underwear. The patient is confused and smells of urine. She is not oriented to place or time, but she knows her name. Pelvic examination shows severe vaginal atrophy. There is a 3-cm, tender laceration and an erythematous, edematous vaginal perineal body. Which of the following is the most likely cause of these findings?

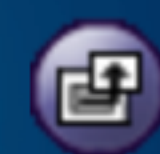
- ☐ A) Acute allergic reaction
- ☐ B) Lichen planus
- ☐ C) Sexual assault
- ☐ D) Squamous cell carcinoma in situ
- ☐ E) Urinary incontinence



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45. A 30-year-old woman comes to the physician for a routine pelvic examination. She has had multiple sex partners. Perianal examination shows two venereal warts. Cervical examination shows no abnormalities. A Pap smear shows epithelial cells with an irregular nuclear contour and a distinct halo around the nucleus with a dense outer rim of cytoplasm. Which of the following is the most likely cause of the cytologic abnormalities?

- ☐ A) Infection with *Chlamydia trachomatis*
- ☐ B) Infection with cytomegalovirus
- ☐ C) Infection with herpes simplex virus 2
- ☐ D) Infection with human papillomavirus
- ☐ E) Invasive cervical carcinoma



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46. Three days after a moderately difficult forceps delivery, a 27-year-old woman, gravida 1, para 1, comes to the physician because of a 3-day history of numbness and tingling of the anterior lower right leg. She also has had buckling of the right knee when she attempts to walk. Epidural anesthesia was administered during labor. After 4 hours of pushing in stirrups during the second stage of labor, spinal anesthesia was administered for the forceps delivery. Examination shows decreased sensation along the anterior, anteromedial, and medial aspect of the right thigh, knee, and upper leg. There is some weakness of the quadriceps femoris muscle. Which of the following is the most likely explanation for these findings?

- ☐ A) Adverse effect of epidural anesthesia
- ☐ B) Adverse effect of spinal anesthesia
- ☐ C) Injury to the femoral nerve
- ☐ D) Injury to the genitofemoral nerve
- ☐ E) Injury to the lateral femoral cutaneous nerve



47. A 28-year-old nulligravid woman has had increasingly severe pelvic pain and dyspareunia over the past 8 months; during menses, the pain worsens and she has occasional episodes of diarrhea. Pelvic examination shows a normal vagina and cervix with no suspicious lesions. The uterus is normal-sized, retroverted, and fixed, and there is induration and nodularity of the posterior wall of the uterus and cul-de-sac. There are no palpable adnexal masses. Which of the following is the most appropriate diagnostic test?

- ☐ A) Measurement of serum CA 125 concentration
- ☐ B) Barium enema
- ☐ C) CT scan of the pelvis
- ☐ D) MRI of the pelvis
- ☐ E) Laparoscopy



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48. A previously healthy, asymptomatic 25-year-old woman comes to the physician for contraceptive advice. She takes no medications. Menses occur at regular 28-day intervals and last approximately 5 days. Her last menstrual period was 20 days ago. Pelvic examination shows a 5-cm mobile mass in the right adnexa; the mass is moderately painful to palpation. Which of the following is the most likely diagnosis?
- ☐ A) Adenocarcinoma of the gastrointestinal tract
 - ☐ B) Adenocarcinoma of the ovary
 - ☐ C) Distended urinary bladder
 - ☐ D) Diverticulitis
 - ☐ E) Ectopic pregnancy
 - ☐ F) Endometrioma
 - ☐ G) Hemorrhagic corpus luteum cyst
 - ☐ H) Lymphoma
 - ☐ I) Tubo-ovarian abscess



49. A 24-year-old primigravid woman at 12 weeks' gestation comes to the physician for her first prenatal visit. She has immune thrombocytopenic purpura and wants to know whether this condition will affect her baby. Examination shows a uterus consistent in size with a 12-week gestation. This patient's fetus is at increased risk for which of the following?

- ☐ A) Chronic maternal anemia
- ☐ B) Fetal platelet destruction
- ☐ C) Fetal renal infarction
- ☐ D) Maternal hypertension causing uteroplacental insufficiency
- ☐ E) Placental infarction



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50. A 27-year-old woman, gravida 2, para 2, comes to the physician because she is concerned that she may be pregnant. She was sexually active with her husband 2 days ago, and they did not use contraception. She wants to prevent an unintended pregnancy. She is breast-feeding their 7-month-old infant. Menses occur at regular 28-day intervals. Her last menstrual period was 1 week ago. She has a 5-year history of migraines and a history of hepatitis B. She was treated for *Chlamydia trachomatis* infection at the age of 17 years. She takes no medications and has no known drug allergies. Vital signs are within normal limits. Physical examination, including pelvic examination, shows no abnormalities. A urine pregnancy test is negative. Which of the following is a contraindication for the use of levonorgestrel as emergency contraception in this patient?

- ☐ A) Breast-feeding
- ☐ B) History of *Chlamydia trachomatis* infection
- ☐ C) History of hepatitis B
- ☐ D) History of migraines
- ☐ E) There are no contraindications

